

Sacrocolpopexy

Patient Information · Lifting the top of the vagina with mesh

WARWIKI

Sacrocolpopexy repairs prolapse of the **top of the vagina** (the apex). It is often used when the top of the vagina drops after a hysterectomy. A soft mesh lifts the top of the vagina and fixes it to a strong ligament at the base of the spine (the sacrum). The vagina keeps its length, so sex is still possible after healing.

About This Procedure

A strip of soft **mesh** is stitched to the front and back of the top of the vagina. The other end is fixed to a strong ligament on the front of the **sacrum** (the bone at the base of the spine). This holds the vagina up in its normal place and keeps its length. It is usually done through small cuts in the belly (laparoscopic or robotic surgery). Some people need a larger, open cut.

Benefits and Risks in Numbers

7 in 10

have no bulge, no prolapse past the opening, and no repeat treatment 3 years later

4 in 100

need more prolapse treatment within 3 years

3–10 in 100

have mesh show through the vaginal wall over 3 to 7 years

Numbers are averages from published studies. Your own risk may differ; ask your team.

About the Mesh

This mesh is placed through the **belly**. It is **different** from mesh placed through the vagina for prolapse. The FDA stopped the sale of that vaginal mesh in the US in 2019. The belly mesh used here is still in use. It is permanent, and problems such as mesh showing through the vaginal wall can appear years later.

Learn the Terms

Apex or vault

The top of the vagina. It can drop after childbirth or a hysterectomy.

Sacrum

The bone at the base of the spine. A ligament on it anchors the repair.

Mesh

A soft, permanent woven material that supports the top of the vagina.

Laparoscopic or robotic

Surgery through a few small cuts, using a camera.

Mesh exposure

When a bit of mesh shows through the vaginal wall. It can cause discharge, bleeding or pain with sex.

Hysterectomy

Removal of the uterus. It is sometimes done at the same time.

Will it hurt? You will be asleep during surgery. Afterward, expect soreness at the small cuts and some gas pain for a few days. Many people go home the same day or the next day. Recovery takes longer after an open cut.

First day

Walk soon after surgery. Your bladder emptying is checked.

First weeks

Return to everyday activity, lifting and exercise at your own pace. Keep stools soft.

Until cleared

Nothing in the vagina and no sex until your team says so, often about 6 weeks.

Later

See your team for checkups. Report any new discharge, bleeding or pain.

How to Get Ready

- The surgery is done under **general anesthesia**. Follow the instructions about eating, drinking and blood thinners.
- Ask whether a **hysterectomy** or a procedure for urine leakage will be done at the same time.
- Stop smoking. Arrange a ride and some help at home.

What Happens During Surgery

- 1 You are asleep and get antibiotics.
- 2 Through small cuts (or one larger cut), the surgeon stitches mesh to the top of the vagina.
- 3 The mesh is fixed to the ligament on the sacrum, lifting the vagina into place.
- 4 The bladder is checked with a small camera. A catheter may stay in for a short time.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Belly pain that is getting worse, or nausea and vomiting that do not stop
- Severe low back pain or pain spreading down a leg, even without a fever
- Later on: discharge, bleeding or pain with sex

Go to an emergency department if:

- You **cannot urinate** for several hours and your bladder feels full or painful
- Bleeding that soaks a pad in an hour, chest pain, or trouble breathing
- You cannot reach the office and are worried

Three Things to Remember

1. Sacrocolpopexy lifts the top of the vagina with mesh fixed to the sacrum. It is one of the longer-lasting prolapse repairs.
2. The belly mesh is permanent. Keep a record of its name, and tell any future doctor you have it.
3. Walk from the first day and return to activity at your own pace.